

The Two Pakistans of Motherhood

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By Dr. Rubina Fareed

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Pakistan has cut its maternal mortality ratio by a third since 2007, yet a woman's chance of a safe delivery still depends on the province she lives in, the household she was born into and the age at which she was married. Drawing on 45 years of obstetric practice and a seat on the National Commission on the Rights of Child, this Insight argues that the divide between the two Pakistans of motherhood is less a shortage of resources than a failure of governance. The mothers at greatest risk are disproportionately girls married before 18, and the state meets most of them for the first time in a labour ward. Because maternal health and child protection were built as separate bureaucracies, a pregnant child can pass through that ward without any record that she is a child. Closing the gap requires three unglamorous administrative changes: completing the legal reform of the marriage age, creating a protected referral from maternity care to child protection, and making the health system answerable for counting births and marriages.

Context

Early in my career I delivered two babies on the same night, in the same city, perhaps 8 kilometres apart. The first mother had attended regular antenatal checkups; her blood pressure and haemoglobin had been tracked for months, and a full surgical team was ready the moment her labour showed signs of complication. The second had never had a single antenatal visit. She reached our hospital after more than a day of obstructed labour, and only because a neighbour owned a motorcycle and knew a doctor's telephone number. We saved her that night, but barely, and I have never forgotten how close "barely" came to "too late". I could describe an almost identical pair of deliveries from last month. The neighbourhoods and the complications change; the architecture of the story has moved far less than it should have.

The national record shows genuine progress, and it deserves to be stated plainly. Between 2007 and 2019 the maternal mortality ratio fell by 33%, from 276 to 186 deaths per 100,000 live births.^{1,2} The decline was concentrated in the countryside. A comparative analysis of the two national surveys found that rural mortality fell by 42%, from 320 to 185, while urban mortality fell by 11%, from 177 to 157, enough in the authors' judgment to close the statistical gap between town and country.¹ The official 2019 survey reports a wider difference, 199 in rural areas against 158 in urban areas, and the two estimates are reported here as competing

figures rather than reconciled.² The sharper divide now runs between provinces: in 2019 Balochistan recorded 298 maternal deaths per 100,000 live births and Sindh 224, against 157 in Punjab.²

Access to care follows the same gradient (Table 1). In 2017-18, 86% of women saw a skilled provider at least once during pregnancy, but only 51% made 4 or more antenatal visits.³ Skilled attendants were present at 84% of urban births and 63% of rural births, and at only 38% of births in Balochistan, where barely a third of women gave birth in a health facility.³ A woman whose hypertension is detected at her first and only antenatal visit, 3 weeks before an unassisted home delivery, has not been failed by biology. She has been failed by a system that treats contact as coverage rather than as the first step in a continuum of usable care.

Table 1. The risks of childbirth rise with distance from the best-served provinces and cities.

Indicator	National	Urban	Rural	Balochistan
Maternal mortality ratio, 2019 (deaths per 100,000 live births) ²	186	158	199	298
At least one antenatal visit with a skilled provider, 2017-18 ³	86%	94%	82%	56%
Births assisted by a skilled provider, 2017-18 ³	69%	84%	63%	38%
Births in a health facility, 2017-18 ³	66%	81%	59%	35%

Source: National Institute of Population Studies and ICF, Pakistan Maternal Mortality Survey 2019 and Pakistan Demographic and Health Survey 2017-18. The antenatal and delivery indicators refer to the most recent live birth or to births in the 5 years preceding the survey.

The second dimension of the divide is age. In 2018, 18% of Pakistani women aged 20 to 24 had been married before 18 and 4% before 15; the rate was 22% in rural areas against 13% in urban areas, and 34% among the poorest households against 7% among the richest.⁴ Some 20.5 million women living in Pakistan were married as children.⁴ At the time of the 2017-18 survey, 8% of women aged 15 to 19 were already mothers or pregnant with a first child.³ Adolescent mothers face higher risks of eclampsia, puerperal endometritis and systemic infections than women in their early twenties, and their infants face higher risks of low birth weight and preterm birth.⁵

The state's capacity to see either problem is limited. Only 42% of children under 5 had their births registered in 2017-18,³ and among the poorest households speaking Balochi, Pashto, Saraiki or Sindhi the rate was around

15%.⁶ The law, by contrast, has moved quickly. Islamabad Capital Territory set 18 as the minimum age of marriage for both sexes in 2025, Balochistan followed in November 2025 and the Punjab Assembly did the same in April 2026, joining Sindh, which legislated in 2013.^{7,8,9} Khyber Pakhtunkhwa still permits the marriage of girls at 16;¹⁰ a draft bill has been vetted by its Law Department but has not yet been placed before the provincial cabinet or assembly.¹¹

Analysis

These figures describe a governance problem more than a resource problem. Pakistan's health workforce is not the weak link. I have spent my professional life among its doctors, midwives and Lady Health Workers, and the exhaustion I have seen in rural health centres belongs to people attempting an impossible task with insufficient tools, not to people who are indifferent. The failure sits one level higher, in how the state designs, funds and, above all, audits its own service delivery.

The distinction between contact and continuity is the clearest illustration. The comparative analysis of the 2007 and 2019 surveys found that, in 2019, women who died were more likely than surviving mothers to have received antenatal care, and its authors call for attention to quality of care at every stage of the continuum rather than at the first point of contact.¹ A visit that identifies danger without securing a skilled birth and a route to emergency obstetric care is recorded as coverage while changing little for the woman concerned. Programmes are judged by the number of women reached, not by whether each woman reached was carried safely through delivery.

My other seat, on the National Commission on the Rights of Child, exposes a dimension that a purely clinical framing misses: a significant share of the mothers at greatest risk in the second Pakistan are not adult women at all. They are children. In Commission meetings, child marriage is discussed, correctly, as a violation of a girl's right to childhood, to education and to consent. In the labour ward, the same girl, now in obstructed labour and silent because she has been taught that a wife does not question her husband's family, is discussed purely as an obstetric case. Nobody in the second room asks the question on which the first room would insist: how old is this patient, and did anyone identify her as a child before she became a patient?

The gap is structural rather than rhetorical. Health information systems record maternal age as a data field, not as a trigger for action. A girl of 15 presenting in labour generates no referral to a child protection mechanism, no inquiry into the marriage and no flag of any kind, by law or by standard operating procedure. A Lady Health Worker is trained to recognise a dangerous pregnancy; she is not trained, resourced or, in most districts, permitted to treat the fact that a patient is 14 as requiring a different response from the fact that she is anaemic. The Child Marriage Restraint Act of 1929 was never written with health facilities in view as a place to detect the marriages it exists to prevent, and the newer statutes still rely chiefly on nikah registrars, the police and the courts. Yet a health facility is one of the few places in which an underage bride reliably enters an official record, because sooner or later almost every pregnancy does. Low birth registration compounds the failure. A state cannot govern what it does not count, and at present it does not count the overlap between two of its weakest indicators.

The reasonable objection is that all of this reduces to money: more rural health centres, more community midwives, more ambulances. In part it does. But I have watched enough well-funded pilot projects quietly expire to distrust any argument that begins and ends with budget lines. The deeper problem is that maternal health and child protection were built as separate bureaucracies that have never been required to communicate. Additional spending channelled through the same two silos would buy more contact without buying more protection.

Implications

Three consequences follow. First, the national maternal mortality ratio overstates how far the second Pakistan has advanced. An average that blends Islamabad with rural Balochistan conceals precisely the populations in which progress has been slowest, and policy judged against that average will keep rewarding gains among women who were already easiest to reach.

Second, every underage pregnancy that reaches a facility is a missed opportunity for enforcement. The new provincial laws raise the legal age of marriage, but a law enforced only on complaint will reach few of the marriages it prohibits, because the families involved rarely complain and the girls concerned seldom can. Without a mechanism that turns a clinical

encounter into a protective one, legislative progress risks remaining largely declaratory.

Third, the cost falls twice on the same girl. She is failed first when the marriage is allowed to proceed and again when she enters a health system with no means of recognising what has happened. The consequences extend to her child, born into the households in which birth registration is lowest, and so likely to begin life as invisible to the state as its mother.

The figures in this Insight are drawn from national surveys, official records and peer-reviewed analysis; the interpretation placed on them, and the conclusions drawn from four decades of clinical practice, are my own judgments and should be read as such.

The Way Forward

This is not a call for another awareness campaign; awareness has never been the scarce resource. It is a call for three administrative changes, none of them glamorous.

First, complete the legal reform. With Sindh, Islamabad Capital Territory, Balochistan and Punjab now at 18, Khyber Pakhtunkhwa is the principal exception, and among the regions where child marriage has been most common.⁴ Its government should place the vetted bill before cabinet and the assembly without further delay, so that a marriage prohibited in Islamabad is not lawful a short drive away across the provincial boundary. Where the new laws are already in force, provincial governments should publish annual data on reported cases and prosecutions, since a statute that is never enforced is an intention rather than a law.

Second, build a mandatory and protected reporting pathway from maternal health facilities to child protection units whenever a patient who is demonstrably under 18 presents. Its purpose is not to punish frightened young mothers; the Punjab reform already provides that children who are parties to a marriage are not to be treated as offenders.⁹ Its purpose is to ensure that the state knows where its failures are concentrated and that each girl is offered protection and support. The pathway should rest on a simple rule written into the standard operating procedures of Lady Health Workers, community midwives and hospital obstetric units, with confidentiality safeguards strong enough that no family is deterred from seeking care.

Third, treat birth and marriage registration as responsibilities of the health system and not only of civil registration. The health system is frequently the only state institution a rural family will willingly enter. Registering each birth at the facility, or at the Lady Health Worker's postnatal visit, and linking the record to the mother's age at delivery would give provincial governments and the National Commission on the Rights of Child a continuous picture of where underage pregnancy is concentrated.

The two Pakistans of motherhood are not an inevitability of geography or poverty, in the way a drought is. They are the product of specific institutional choices about which data to collect, which laws to harmonise and which bureaucracies to compel to cooperate, and they can be unmade by equally specific choices. The girl who arrives in obstructed labour at 14 is not best understood, first, as a maternal health statistic. She is a child whom the state had promised, on paper, to protect, and whom it has already failed twice by the time she reaches the operating table: once when it allowed the marriage, and again when it built a health system with no means of noticing. After 45 years, I no longer believe that the second Pakistan of motherhood will catch up with the first if we are simply patient. Closing the gap will take less money than is commonly assumed, and more institutional honesty than has so far been supplied.

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About the Author

Dr. Rubina Fareed is an obstetrician and gynaecologist who has practised since 1980 and serves as Member for Islamabad Capital Territory on the National Commission on the Rights of Child (NCRC), Government of Pakistan. A graduate of Khyber Medical College, University of Peshawar, she holds a Diploma in the Management of Obstetric Emergencies and Trauma from the United Kingdom and runs a clinic and assisted reproduction centre in Islamabad. She served as Medical Director of the Melody Relief and Rehabilitation Centre after the 2005 earthquake and has chaired the caring committees of the Family Protection and Rehabilitation Centre for Women and the National Cell for the Protection of Children. Her honours include the Mohtarma Fatima Jinnah Gold Medal and the National Human Rights Award.